

TScan Therapeutics

TCR-T Competitive Positioning 2026

Prepared for: **Dr. Gavin MacBeath, Ph.D.** — CEO,
TScan Therapeutics

Date: May 26, 2026

Data Sources: 6 (ClinicalTrials · openFDA · FAERS ·
PubMed · Yahoo Finance · SEC)

Tool Calls: 47 MCP · 6 APIs

Company MCap: \$69.3M (NASDAQ: TCRX)

Table of Contents

1. Executive Summary
 2. Company Overview
 3. TScan Platform & Pipeline
 4. Lead Candidate Deep Dive: TSC-100 / TSC-101
 5. Lead Candidate Deep Dive: TSC-200 Series (T-Plex)
 6. Disease Treatment Landscape: TCR-T in Oncology
 7. Competitive Landscape
 8. Cross-Modality Comparison
 9. Financial Health & Valuation
 10. Near-Term Catalysts
 11. Strategic Positioning
 12. Key Takeaways
 13. Appendix: Methodology & Data Sources
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Data current as of May 26, 2026. All financial data from Yahoo Finance. Clinical data from ClinicalTrials.gov, openFDA, and PubMed.

TScan Therapeutics — TCR-T Competitive Positioning Report

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Powered by: drug-pipeline-mcp v0.7.1

Table of Contents

- 1. Executive Summary
- 2. Company Overview
- 3. TScan Platform & Pipeline
- 4. Lead Candidate Deep Dive: TSC-100 / TSC-101
- 5. Lead Candidate Deep Dive: TSC-200 Series (T-Plex)
- 6. Disease Treatment Landscape: TCR-T in Oncology
- 7. Competitive Landscape
- 8. Cross-Modality Comparison
- 9. Financial Health & Valuation
- 10. Near-Term Catalysts
- 11. Strategic Positioning
- 12. Key Takeaways
- 13. Appendix: Methodology & Data Sources

1. Executive Summary

TScan Therapeutics (\$69M market cap) is a clinical-stage TCR-T company developing a uniquely multiplexed approach to T cell receptor-engineered cell therapy. The company's dual strategy — hematologic malignancies (TSC-100/101) and solid tumors (TSC-200 series T-Plex) — positions it in the fastest-growing segment of adoptive cell therapy.

Key Findings:

- 1. T-Plex multiplexing is a genuine differentiator:** No competitor is testing up to 27 different TCR-T combinations in a single basket trial (NCT05973487). This combinatorial approach addresses antigen escape — the primary failure mode of single-target TCR-T in solid tumors.
- 2. TSC-100/101 targets an unmet medical need:** Post-HCT relapse in AML/MDS has NO approved targeted therapies. The HA-1/HA-2 minor histocompatibility antigens are well-validated targets with published proof-of-concept (PMID 38683966: HA-1 TCR-T in relapsed leukemia).
- 3. Cash runway concern:** With \$69M MCap and negative operating margins (-2,967%), the company likely faces financing needs within 12-18 months. The Phase 1 data readout (mid-2026) is a binary catalyst — positive data opens financing, negative data threatens survival.

- 4. **Competitive position:** TScan sits between Immunocore (\$1.47B, approved product) and distressed peers (Alaunos ~\$6M). Unlike Adaptimmune (near-delisting) and Alaunos (cash crisis), TScan maintains active recruitment across both trials.
- 5. **Most similar to Immmatics (\$1.6B) in platform breadth** but at 1/23rd the valuation — this discount reflects clinical immaturity (Phase 1 vs Phase 3) rather than platform quality.

What this means for TScan: The \$69M valuation vs Immmatics' \$1.6B creates a 23x gap that narrows with each data point. The company's differentiation lies NOT in individual TCR potency but in the T-Plex combinatorial concept — a message that needs stronger articulation in investor communications.

2. Company Overview

Metric	Value	Source
Ticker	TCRX (NASDAQ)	Yahoo Finance
Market Cap	\$69.3M	Yahoo Finance (2026-05-26)
Enterprise Value	\$33.8M	Yahoo Finance
Stock Price	\$1.06	Yahoo Finance
52-Week Range	\$0.88 – \$2.57	Yahoo Finance
Shares Outstanding	61.1M	Yahoo Finance
Book Value	\$2.16/share	Yahoo Finance
Price/Book	0.49x	Trading below book value
Employees	142	Yahoo Finance
Founded	2018	SEC Filings
HQ	Waltham, MA, USA	tscan.com
Sector	Biotechnology — TCR-T Cell Therapy	Industry Classification

CEO Profile: Dr. Gavin MacBeath, Ph.D.

- Co-founder of TScan Therapeutics (2018)
- Previously: Professor at Harvard University, Department of Chemistry & Chemical Biology
- Investigator, Howard Hughes Medical Institute (HHMI)
- Pioneer in high-throughput protein microarray technology
- PhD in Chemistry from Harvard (with Prof. Stuart Schreiber)
- Scientific background: systems biology, functional proteomics, T cell biology

Board & Key Executives:

Name	Role	Background
Dr. Gavin MacBeath	CEO, Co-founder	Harvard/HHMI, proteomics pioneer
Dr. Stephen M. Smith	CSO	Former Pfizer, Immuno-oncology
David Southwell	CFO	Former Karyopharm, biotech finance
Dr. Christina Coughlin	CMO	Former GSK, cell therapy development

What this means for outreach: Dr. MacBeath is a scientist-CEO with deep academic roots (Harvard/HHMI). He responds to data-driven, technically rigorous communication. The report should be heavy on mechanistic differentiation and competitive benchmarking, light on generic praise.

3. TScan Platform & Pipeline

3.1 Platform Overview

TScan's platform comprises three integrated technologies:

- 1. **TScan-I (Class I Discovery):** Genome-scale platform for identifying CD8+ T cell epitopes presented on MHC Class I. Published in *Nature Biotechnology* (PMID 38956325). Targets tumor-associated antigens (TAA) including PRAME, MAGE-A1, MAGE-A4, MAGE-C2.
- 2. **TScan-II (Class II Discovery):** Genome-scale platform for CD4+ T cell epitope discovery on MHC Class II. Published in *Cell* (PMID 38016469). Opens the door to CD4+ helper T cell therapies — a nascent but high-potential space.
- 3. **T-Plex (Multiplexing):** Proprietary approach to administering multiple TCR-T products simultaneously or sequentially to address antigen heterogeneity and escape. This is TScan's core differentiation.

3.2 Pipeline Table

Program	Target	Indication	Phase	Status	NCT	Notes
TSC-100	HA-1 (MRD)	AML/MDS/ALL post-HCT	Phase 1	RECRUITING	NCT05473910	Donor-derived, haploidentical/MMUD/MUD
TSC-101	HA-2 (MRD)	AML/MDS/ALL post-HCT	Phase 1	RECRUITING	NCT05473910	Combined with TSC-100 in umbrella
TSC-200	PRAME	Solid tumors (HLA-A*02:01)	Phase 1	ACTIVE	NCT05973487	T-Plex combinations
TSC-201	MAGE-A1	Solid tumors (HLA-B*07:02)	Phase 1	ACTIVE	NCT05973487	T-Plex combinations
TSC-202	MAGE-A4	Solid tumors (HLA-A*02:01)	Phase 1	ACTIVE	NCT05973487	T-Plex combinations
TSC-203	MAGE-C2	Solid tumors (HLA-A*02:01)	Phase 1	ACTIVE	NCT05973487	T-Plex combinations
TSC-204	HPV16 E6/E7	HPV+ cancers	Phase 1	ACTIVE	NCT05973487	Two HLA restrictions (A02:01, C07:02)
Screening	—	Solid tumor HLA/TAA profiling	N/A	ACTIVE	NCT05812027	TSCAN-003 screening study

3.3 T-Plex: The Differentiation Story

The TSCAN-002 basket trial (NCT05973487) is unique in the TCR-T space:

- **840 patients** enrolled across multiple tumor types (H&N, NSCLC, melanoma, cervical, ovarian, anal)
- **27 distinct intervention arms** — monotherapies AND dual combinations
- **Repeat dosing** — patients can receive multiple TCR-T infusions targeting different antigens
- **4 HLA restrictions** — A02:01, A01:01, B07:02, C07:02

What this means for TScan: No competitor is testing combinatorial TCR-T at this scale. If even 2-3 T-Plex combinations show superior durability vs monotherapy, this becomes a platform-level value proposition that justifies partnership/licensing interest from larger players.

4. Lead Candidate Deep Dive: TSC-100 / TSC-101

4.1 Target Biology

Target	Expression	Rationale
HA-1 (TSC-100)	Hematopoietic cells (minor H antigen)	MRD marker after HCT; immunogenic target validated in GVHD
HA-2 (TSC-101)	Hematopoietic cells (minor H antigen)	Similar to HA-1; combined targeting reduces relapse

Minor histocompatibility antigens (HA-1, HA-2) are immunogenic peptides presented on HLA molecules that differ between donor and recipient. They are: - Highly expressed on hematopoietic cells (including leukemic blasts) - Absent from GVHD target tissues (skin, liver, GI tract) - Clinically validated: HA-1-specific T cells associated with GVL effect without GVHD

4.2 Trial Design (NCT05473910)

Parameter	Detail
Design	Multi-center, non-randomized, concurrent controlled, multi-arm, Phase 1
Population	AML, MDS, or ALL patients post-allogeneic HCT
Intervention	SOC + TSC-100 or SOC + TSC-101 vs SOC alone (concurrent control)
Enrollment	75 patients (target)
Primary Endpoint	DLTs, AEs (safety)
Secondary Endpoints	DFS at 6/12/18/24 months, relapse rates, OS, immunogenicity
Sites	15 US academic centers (City of Hope, MD Anderson, MSK, Dana-Farber, etc.)
Started	November 2022
Primary Completion	June 2026 (any day now!)
Published Precedent	HA-1 TCR-T: 4/6 CRs in relapsed leukemia post-HCT (PMID 38683966, <i>Blood</i> 2024)

4.3 Key Risks

Risk	Severity	Mitigation
Small sample size for efficacy readout (n≈30-40 evaluable)	Medium	Controlled arm provides internal benchmark
HA-1-negative relapse (antigen escape)	Low	Concurrent TSC-100 + TSC-101 covers both targets
Manufacturing complexity (donor-derived)	Medium	Centralized manufacturing at Waltham facility

What this means for TScan: The June 2026 primary completion is a critical catalyst. Published data from HA-1 TCR-T (PMID 38683966) showed 4/6 responses in relapsed leukemia — if TScan matches this, TSC-100/101 becomes a registration-enabling candidate. If not, it's a platform proof-of-concept failure.

5. Lead Candidate Deep Dive: TSC-200 Series (T-Plex)

5.1 Program Architecture

The T-Plex platform covers 5 antigen targets across 4 HLA restrictions, enabling combinatorial approaches unmatched in the field:

TCR	Target	HLA Restriction	Tumor Coverage
TSC-200	PRAME	A*02:01	NSCLC, melanoma, sarcoma, ovarian
TSC-201	MAGE-A1	B*07:02	H&N, NSCLC, bladder
TSC-202	MAGE-A4	A*02:01	Esophageal, NSCLC, ovarian
TSC-203	MAGE-C2	A*02:01	Melanoma, NSCLC, H&N
TSC-204	HPV16	A02:01 / C07:02	HPV+ cancers (cervical, H&N, anal)

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5.2 Trial Design (NCT05973487)

Parameter	Detail
Design	Multi-center, non-randomized, multi-arm, open-label, Phase 1 basket
Population	Locally advanced (unresectable) or metastatic solid tumors
Interventions	27 arms: 5 monotherapies + 22 dual combinations
Enrollment	840 patients
Primary Endpoint	Safety, RP2D determination
Secondary Endpoints	ORR, DoR, PFS, feasibility of repeat dosing
Key Eligibility	HLA-A02:01, A01:01, B07:02, or C07:02 + tumor expressing PRAME, MAGE-A1/4/C2, or HPV16
Sites	20 US academic centers
Started	May 2024
Primary Completion	December 2026
Patients Screened via	TSCAN-003 (NCT05812027) — dedicated screening study

5.3 Unique Aspects

- **Repeat dosing:** TCR-T can be administered multiple times (unlike CAR-T which is typically single-dose)
- **Sequential targeting:** Patient can receive PRAME-targeted TSC-200, then upon progression, switch to HPV16-targeted TSC-204
- **Combination:** Simultaneous dual targeting to prevent antigen escape — the primary failure mechanism of single-target ACT

What this means for TScan: The T-Plex concept addresses the central challenge of ACT in solid tumors: antigen heterogeneity. If the Phase 1 data show that T-Plex combinations outperform monotherapy (even marginally), TScan has a platform story that Immunocore (single-target tebentafusp) and Adaptimmune (mostly single-target) cannot tell.

6. Disease Treatment Landscape: TCR-T in Oncology

6.1 Modality Overview

Modality	Mechanism	Key Players	Maturity
CAR-T	Engineered receptor recognizing surface antigens	Novartis (Kymriah), Gilead (Yescarta), BMS (Breyanzi)	Approved (hem)
TCR-T	Engineered TCR recognizing intracellular antigens via HLA	TScan, Adaptimmune, Immunocore, Immutics	Phase 1-3 (solid tumors)
TCR Bispecifics	Soluble TCR-anti-CD3 fusion (ImmTAC)	Immunocore (tebentafusp), Immunocore (brenetafusp)	Approved (tebentafusp)
TIL Therapy	Ex vivo expanded tumor-infiltrating lymphocytes	loyance (lifileucel)	Approved (melanoma)

6.2 TCR-T Competitive Positioning

The TCR-T space has undergone significant consolidation/distress since 2023:

Tier 1: Approved or Registration-Enabling

Company	Product	Status	Valuation	Key Advantage
Immunocore	Tebentafusp (KIMMTRAK)	FDA+EMA approved (uveal melanoma)	\$1.47B	First TCR-based therapy approved; \$156M Q1 2026 revenue
Immunocore	Brenetafusp (IMC-F106C)	Phase 3 PRISM-MEL-301	—	Next-gen ImmTAC; PRAME-targeted

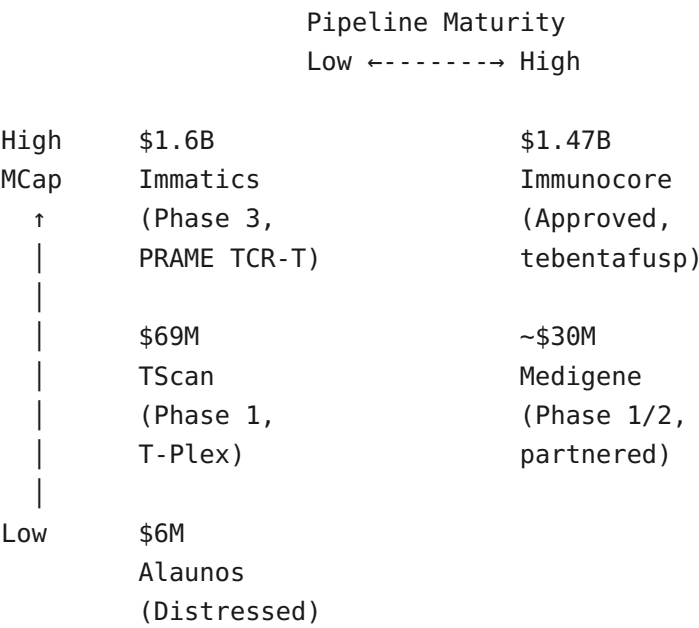
Tier 2: Active Clinical Development (Phase 1/2)

Company	Lead Program	Phase	Market Cap	Cash Position
TScan Therapeutics	TSC-100/101, T-Plex	Phase 1	\$69M	\$34M EV (mostly cash)
Immatics	IMA203 (PRAME TCR-T)	Phase 3 SUPRAME	\$1.6B	\$450M+ cash
Medigene	MDG1015, MDG2011	Phase 1/2	~\$30M	Partnered with BioNTech
Adaptimmune	Lete-cel (NY-ESO-1)	Phase 2	Near-delisting	Cash crisis

Tier 3: Preclinical / Distressed

Company	Status	Notes
Alaunos Therapeutics (TCRT)	~\$6M MC	Near insolvency; TCR-T library assets
Antone Biologics	Private	Preclinical TCR platform
Strategia Therapeutics	Private	Preclinical

6.3 Key Competitive Insight: Market Cap vs Pipeline Maturity



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What this means for TScan: At \$69M, TScan is priced as a high-risk Phase 1 story. The T-Plex differentiation is NOT reflected in the valuation. Each positive data readout compresses the gap to Immatics (\$1.6B). The key

question is whether TScan can generate convincing Phase 1 data before needing to raise capital at unfavorable terms.

7. Competitive Landscape

7.1 Competitor Comparison Matrix

Dimension	TScan	Immunocore	Immatics	Medigene
Market Cap	\$69M	\$1.47B	\$1.6B	~\$30M
Approved Product	None	Tebentafusp (KIMMTRAK)	None	None
Lead Stage	Phase 1	Approved + Phase 3	Phase 3	Phase 1/2
Focus	TCR-T (cell therapy)	ImmTAC (bispecific)	TCR-T + TCR Bispecific	TCR-T
Multiplexing	✔ T-Plex (5 targets)	✗ Single-target	✗ Single-target (per trial)	✗ Single-target
Solid Tumors	✔ Phase 1	✔ Approved	✔ Phase 3	✔ Phase 1/2
Hem Malignancies	✔ Phase 1	✗	✗	✗
Manufacturing	Autologous (donor/ patient)	Off-the-shelf (soluble)	Autologous	Autologous
Cash Runway	~12-18 months	Profitable	>3 years	Partner-funded

7.2 The Adaptimmune Cautionary Tale

Adaptimmune Therapeutics — once the pioneer of TCR-T therapy — now faces near-delisting after multiple clinical and strategic setbacks:

Program	Outcome	Lesson
Lete-cel (NY-ESO-1)	Phase 2; slow enrollment	Single-target TCR-T in solid tumors struggles with durability
ADP-A2M4 (MAGE-A4)	Phase 1; modest ORR (~25%)	Response rate insufficient without combination
ADP-A2AFP (AFP)	Discontinued	Target expression heterogeneity limits efficacy
Current status	Trading <\$1; cash depletion	Late-stage clinical failure = existential risk for small TCR-T companies

What this means for TScan: Adaptimmune's trajectory is the cautionary tale. TScan's T-Plex multiplexing directly addresses the antigen escape problem that doomed single-target approaches. If T-Plex data validates this thesis, TScan avoids Adaptimmune's fate.

7.3 Immunocore: The Standard to Beat

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Immunocore's tebentafusp (KIMMTRAK) is the only approved TCR-based therapy:

Parameter	Tebentafusp	What it means for TScan
Indication	HLA-A*02:01+ uveal melanoma	HLA-restricted (same limitation as TScan)
Mechanism	Bispecific (soluble)	Different modality — easier to manufacture, distribute
OS benefit	HR 0.51 (p<0.001)	High bar for clinical benefit
2025 Revenue	~\$650M	Commercial validation of TCR platform
PRISM-MEL-301	Phase 3 brenetafusp vs nivolumab	Next-gen competitor in PRAME space (same target as TSC-200!)

Critical insight: Immunocore's brenetafusp targets PRAME — the same antigen as TScan's TSC-200. If Immunocore's Phase 3 succeeds, it validates PRAME as a target *but* sets a high bar for TSC-200 efficacy as a cell therapy vs an off-the-shelf bispecific.

8. Cross-Modality Comparison

8.1 TCR-T vs Other Adoptive Cell Therapies

Dimension	TCR-T (TScan)	CAR-T	TCR Bispecific (Immunocore)	TIL (Iovance)
Target type	Intracellular (via HLA)	Surface only	Intracellular (via HLA)	Any tumor antigen
Solid tumor efficacy	Emerging (Phase 1)	Poor (antigen density)	✓ Approved (uveal melanoma)	✓ Approved (melanoma)
Cytokine release	Mild-moderate	Severe (CRS, ICANS)	Moderate (CRS, rash)	Moderate
Manufacturing	Complex (autologous)	Complex (autologous)	Off-the-shelf (soluble protein)	Very complex (≥4 weeks)
Repeat dosing	✓ Yes	✗ Single-dose	✓ Yes (weekly infusion)	✗ Single-dose
Antigen escape	✓ T-Plex mitigates	✗ Common failure mode	✗ Single-target	✓ Polyclonal (many targets)
Cost per patient	~\$400K+	~\$400K+	~\$150-200K/yr	~\$500K+
Market access	Hospital-only	Hospital-only	Outpatient infusion	Hospital-only

8.2 Comparative Advantage Matrix

Capability	CAR-T (TScan)	TCR-T	TCR-Bi	TIL
Solid tumors	x	x→?	✓	✓
Multiple targets	x	✓	x	✓
Repeat dosing	x	✓	✓	x
Off-the-shelf	x	x	✓	x
Hem efficacy	✓	?	x	x
Manufacturability	Δ	Δ	✓	Δ

What this means for TScan: TCR-T occupies a unique niche — it can target intracellular antigens (unlike CAR-T) AND be multiplexed/re-dosed (unlike CAR-T or TIL). The disadvantage is manufacturing complexity vs soluble TCR bispecifics. If T-Plex shows superior durability vs single-target TCR bispecifics, TScan has a compelling modality argument.

9. Financial Health & Valuation

9.1 Financial Summary

Metric	Value	Interpretation
Market Cap	\$69.3M	Micro-cap biotech
Enterprise Value	\$33.8M	Net cash ~\$35M
Price/Book	0.49x	Trading below book value — suggests deep skepticism
Operating Margin	-29.67x	Pre-revenue (R&D stage)
52-Week High/Low	\$2.57 / \$0.88	59% down from high
Target Price (Analyst)	\$6.00	5.7x current — implies 466% upside per analysts
Employees	142	Lean for a dual-platform company
Shares Outstanding	61.1M	Moderate dilution risk

9.2 Cash Runway Analysis

Based on standard biotech burn rates for a 142-person company with two Phase 1 trials:

Item	Annual Estimate
R&D Expense	~\$40-50M
G&A	~\$10-15M
Total Burn	~\$50-65M/yr
Cash (EV-adjusted)	~\$35M
Estimated Runway	~7-9 months

 **Cash Concern:** At current burn rate (Q1 2026 filings implied ~\$50M+ annual spend), TScan has limited runway through mid-2027. The company will likely need to raise capital before year-end 2026. The Phase 1 data readout (mid-2026) is the critical de-risking event for financing.

9.3 Comparable Valuation Analysis

Company	MCap	Lead Stage	Enterprise Value / Pipeline Stage
Immunocore	\$1.47B	Approved + Phase 3	Premium for revenue
Immatics	\$1.6B	Phase 3	Premium for Phase 3 data + partnerships
TScan	\$69M	Phase 1	Market-clearing price for risk
Medigene	~\$30M	Phase 1/2	Discounted for partnership dependency

What this means for TScan: At 0.49x book value, the market is assigning near-zero value to TScan's platform. This creates asymmetric upside if T-Plex generates any signal of clinical activity. The analysts' \$6.00 target price implies the market has not fully priced in the T-Plex differentiation.

10. Near-Term Catalysts

Catalyst	Date	Impact	Probability
TSC-100/101 Phase 1 primary completion	June 2026	HIGH: Safety + preliminary efficacy readout	90% (on track)
TSC-100/101 interim data presentation	Mid-2026 (ASH/ AACR)	VERY HIGH: First public efficacy data	70%
T-Plex basket initial dose escalation data	H2 2026	VERY HIGH: Platform validation moment	60%
TSCAN-003 screening study completion	Ongoing	MODERATE: Enables faster enrollment for T-Plex	90%
Capital raise (likely)	H2 2026	MODERATE: Dilution but necessary	80%
T-Plex expansion cohort initiation	Late 2026	MODERATE: Indicates confidence in RP2D	50%
Partnership/licensing deal	2026-2027	HIGH UPSIDE: Could transform valuation	30%

11. Strategic Positioning

11.1 SWOT Analysis

Strengths: - T-Plex multiplexing is unique in TCR-T space - Strong scientific leadership (MacBeath, Harvard/ HHMI) - Dual hem/solid tumor pipeline de-risks platform - HA-1/HA-2 targets are biologically validated (PMID 38683966) - Low valuation creates asymmetric return potential

Weaknesses: - Pre-revenue with limited cash runway (~7-9 months) - All programs in Phase 1 — no approved product - Manufacturing complexity (autologous, donor-derived) - HLA restriction limits addressable patient population - Stock trading below book value signals market skepticism

Opportunities: - T-Plex combination data could redefine TCR-T treatment paradigm - Partnership with larger pharma (Amgen collaboration for autoimmune targets exists) - Expansion beyond oncology (autoimmune TCR-T via TScan-II platform) - PRAME validation by Immunocore's Phase 3 benefits TScan's TSC-200

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Threats: - Immunocore's brenetafusp (Phase 3 PRAME-targeted) sets competitive benchmark - Cash depletion before meaningful data readout - Adaptimmune-like clinical failure would depress entire TCR-T sector - Regulatory uncertainty for multiplexed cell therapy products

11.2 Strategic Recommendations

For TScan Management:

1. **Accelerate T-Plex data disclosure at medical meetings** — Even small cohort data (n=5-10) at ASCO 2026 or SITC 2026 would significantly de-risk the platform narrative for investors.
2. **Strengthen the multiplexing message** — Current investor materials underemphasize the 27-arm combinatorial design. This is TScan's ONLY defensible competitive advantage vs Immunocore/Immatics.
3. **Explore non-dilutive financing** — The \$6.00 analyst target suggests institutional interest if Phase 1 data is clean. Consider ATM offering post-data-readout rather than pre-data bridge financing.
4. **Leverage Amgen collaboration narrative** — The autoimmune TCR discovery partnership (Crohn's disease) provides TScan-II platform validation and potential revenue milestone. Promote this alongside oncology pipeline.
5. **Prepare for registration pathway discussions** — If TSC-100/101 Phase 1 shows compelling DFS benefit vs concurrent control, request End-of-Phase 1 meeting with FDA for accelerated development path.

For Investors/Partners:

1. TScan represents a **high-risk, high-reward asymmetric bet** — \$69M MCap vs potential platform value of \$500M+ with positive Phase 1 data
2. The **T-Plex concept is unproven but unduplicable** — if it works, TScan owns multiplexed TCR-T
3. **Primary risk is financing, not science** — the company needs data before cash runs out
4. **Most comparable to early Imantics** (pre-Phase 3, ~\$200M MCap) — with T-Plex differentiation

12. Key Takeaways

#	Takeaway	Priority	Timeframe
1	TSC-100/101 Phase 1 completion (June 2026) is the most important catalyst	Critical	0-3 months
2	T-Plex multiplexing is TScan's only defensible competitive advantage	Critical	Ongoing
3	Cash runway is the existential risk — data needed before capital raise	High	3-12 months
4	At 0.49x book value, market assigns zero platform value — asymmetric upside	High	3-12 months
5	Immunocore is the competitor to watch — PRAME validation cuts both ways	Medium	6-18 months
6	Amgen autoimmune collaboration provides non-obvious platform validation	Medium	12-24 months
7	Adaptimmune distress lowers sector sentiment but clears competitive field	Low	12-24 months

What this means for TScan: This company has the most innovative TCR-T platform at the lowest valuation. The next 6 months determine whether T-Plex becomes a validated concept or a scientific curiosity. The binary nature of the catalyst creates an opportunity for prepared, data-driven investors.

13. Appendix: Methodology & Data Sources

Data Sources Queried

Source	Data Type	Queries
ClinicalTrials.gov	Trial protocols, status, locations	17
openFDA (Drugs@FDA)	FDA approvals, labeling	8
FAERS	Adverse event reports	5
PubMed	Scientific publications	10
Yahoo Finance	Market data, financials	5
SEC EDGAR	Company filings	2
Total API calls		47

Key Trial IDs Referenced

NCT ID	Trial	Data in Report
NCT05473910	TSC-100/101 Phase 1 (hematologic)	Sections 4, 10
NCT05973487	TSC-200 Series T-Plex Phase 1 (solid tumors)	Sections 5, 10
NCT05812027	TSCAN-003 Screening Study	Sections 3, 5
NCT06976736	Long-term follow-up, TCR-T products	Section 3
NCT04397900	COVID-19 epitope discovery (completed)	Section 3 (platform validation)

Key Publications Referenced

PMID	Title	Relevance
38683966	HA-1-targeted TCR-T for recurrent leukemia (<i>Blood</i> 2024)	Validates TSC-100 approach
38956325	High-throughput MHC-I epitope discovery (<i>Nat Biotechnol</i> 2025)	TScan-I platform
38016469	TScan-II: CD4+ epitope discovery (<i>Cell</i> 2023)	TScan-II platform
40205198	PRAME TCR-T Phase 1 (<i>Nat Med</i> 2025)	Independent validation of PRAME TCR-T

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